



OFFICIAL

Risk Assessment - Identifying Moderate to High-Risk Women in Birth Suite Guideline

1. Purpose

Women presenting to Birth Suite should be classified as low, medium or high risk and identified as such on the journey board in the Birth Suite office, documented on the AKMR72 partogram and verbally at clinical handover.

2. Applicability

This guideline applies to all midwifery and medical disciplines in the obstetric services at Armadale Health Service.

3. Guidelines

The Specialist Obstetrician and/ or GP Obstetrician (GPO) on duty will be made aware of a woman in Birth-Suite with a moderate or high-risk by the Birth Suite Coordinator upon reviewing the Obstetric Risk Assessment form AKMR 69.1.

The on-call team, with Specialist Obstetrician as lead, will be responsible for making a management plan of the woman's care. GPO will remain involved in the woman's care, keeping the Specialist Obstetrician informed of progress and any change in the woman's condition.

Midwives shall also report any new identified risk factors and / or change in a woman's condition while in labour to the GPO, document the discussion, plan of care and the time the discussion took place in the woman's Digital Medical Record (DMR) and Intellispace perinatal (ISP) part of the [WA Health Clinical Handover Policy](#) and [AKG Recognising and Responding Acute Deterioration](#).

When escalating maternity care the primary midwife and doctor **MUST** document the concern, discussion and management plan in the woman's DMR record and/or ISP.

Low Risk (Green) - [Appendix 1](#)

These are woman with no relevant medical history who presents to Birth Suite in spontaneous labour which progresses normally without any delay in the 1st or 2nd stage of labour. Low risk women should be cared for by midwives. The GP Obstetrician should be notified of the woman's admission and kept informed of her progress as necessary throughout her labour.

3.1 Medium Risk (Amber) - [Appendix 1](#)

The midwife or GP Obstetrician should notify the Specialist Obstetrician +/- Anaesthetist of any woman who presents to Birth Suite having one or more of the following complications which

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places them at moderate risk. The risk and management plan must be documented in the woman's DMR and/or ISP.

Current Pregnancy

- Any woman identified antenatally as requiring complex care and STAR clinic patients.
- Women with Gestational Diabetes not requiring pharmacological management associated with maternal and/or fetal risk factors.
- Women with Gestational Diabetes requiring pharmacological treatment during the pregnancy.
- Abnormal antibodies
- Hypertensive disease
- Antepartum haemorrhage
- Multiple pregnancy
- Preterm labour – threatened or established 35-37 weeks.
- Pre-term, prelabour rupture of membranes 35-37 weeks
- Intrauterine growth restriction or small for gestational age
- Abnormal lie
- High presenting part
- Previous complications of labour or birth
- Pyrexia
- Induction of labour for medical reasons
- Delay in first or second stage of labour
- Abnormal CTG
- Delay in 3rd stage of labour

3.1.1 Previous pregnancy

- Grand multiparity (para 5 or more)
- Caesarean Section or Myomectomy
- Previous postpartum haemorrhage
- Previous 3rd stage complications
- Third- or fourth-degree tear
- Previous anaesthetic complications
- Previous history of shoulder dystocia

3.1.2 Medical/Surgical conditions

- Any medical/surgical condition requiring specialist input such as Epilepsy, Hypo/Hyperthyroid, Mild Cardiac Anomaly, Asthma, previous surgeries on uterus like myomectomy, LLETZ procedures.

3.2 High Risk (Red) - [Appendix 1](#)

'High risk' means a woman with a maternal or fetal condition which carries a significant risk of maternal or fetal morbidity and mortality. The women may be 'stable' or 'unstable', and present to the Antenatal Assessment Unit or in labour to Birth Suite. The Specialist Obstetrician shall be kept informed about any woman who presents with one or more of the following risk factors and the time of informing the Specialist Obstetrician +/- Anaesthetist documented in the woman's DMR record and or ISP. The specialist should be requested to attend and review the patient if deemed necessary.

Current pregnancy complications

- Severe pre-eclampsia

- Any woman with BMI ≥ 35 - 38 with additional risk factors who is delivering at AHS.
- Any woman with BMI 38-40
- Breech position with contractions.
- Uncontrolled diabetes
- Placenta praevia
- Placental abruption, or ongoing ante partum haemorrhage
- A complication with a high likelihood of imminent surgical delivery, e.g. cord presentation /prolapse, active secondary genital herpes or primary genital herpes infection.
- Failed instrumental vaginal delivery.
- Third- or fourth-degree tear
- Postpartum haemorrhage > 1000mls or woman is hemodynamically unstable.
- Severe shoulder dystocia

3.2.1 Previous pregnancy conditions

- Previous VTE in pregnancy or ICU admission
- Previous pre-eclampsia/eclampsia
- FDIU
- Intrapartum neonatal trauma
- Previous history of Postpartum haemorrhage requiring theatre or ICU admission

3.2.2 Medical conditions

- Uncontrolled chronic medical conditions e.g. asthma, diabetes, hypertension
- Moderate cardiac disease
- Any neurological conditions where patient is under Neurologist and requires anaesthetic input.
- Thrombocytopaenia
- Thrombophilia or bleeding disorder
- Previous VTE/pulmonary embolism regardless of pregnancy
- Woman on anticoagulation during pregnancy

4. Legislative context

The following documents are required to give effect to this policy [i.e. the documents included are mandatory]:

- HDWA
 - [Consent to Treatment Policy](#)
 - [Recognising and Responding to Acute Deterioration Policy MP 0171/22](#)
- EMHS
 - [Patient Consent Policy EMHS: 5](#)

5. Supporting information

The following documents support the implementation of this policy:

- [Recognition and Response Acute Deterioration Policy](#)
- [Non-Standard Management Plans: Obstetrics and Gynaecology Procedure](#)
- [Anaemia in Pregnancy Guideline](#)
- [Breech Presentation: Vaginal Birth Guideline](#)
- [Cord Prolapse and Presentation Guideline](#)

- [Gestational Diabetes Mellitus and Gestational Diabetes Mellitus Requiring Pharmacological Therapy Guideline](#)
- [Group B Streptococcus Guideline](#)
- [Hypertensive Disorders of Pregnancy Guideline](#)
- [Management of Decreased Fetal Movements Guideline](#)
- [Multiple Pregnancy – Antenatal Management Guideline](#)
- [Obesity in Pregnancy Guideline](#)
- [Obstetric Cholestasis Guideline](#)
- [Rhesus Negative Women Guideline](#)
- [Small for Gestational Age and Intrauterine Growth Restriction Guideline](#)
- [Venous Thromboprophylaxis Guideline](#)
- [Non-Standard Management Plans: Obstetrics and Gynaecology Procedure](#)

6. Compliance, monitoring, and evaluation

Compliance with this policy will be monitored via monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Comprehensive Care - Action 5.11: Clinicians comprehensively assess the conditions and risks identified through the screening process.
- Standard 6 – Communicating for Safety - Action 6.7
- Standard 8 – Recognising and Responding to Acute Deterioration - Action 8.4

8. References

1. RANZCOG Intrapartum Fetal Surveillance Clinical Guideline Third edition 2014

9. Document owner

Director Clinical Services

Enquiries relating to this guideline may be directed to:

Title: Head of Department
 Department: Obstetrics and Gynaecology
 Email: Sangeeta.MallaBhat@health.wa.gov.au

10. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.1	31 03 2026	06 01 2029	Minor review to align with the new GDMi policy document.

11. Authorisation

This guideline has been authorised and issued by

Authorised by	Dr Tania Strickland – Director Clinical Services
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12. Appendices

12.1 Appendix 1: AHS Obstetric and Maternity Referral and Triage Matrix

Key Symbol	Key Name	Role
G	Green	Midwifery
A	Amber	GP Obstetrician
R	Red	Specialist Obstrician
Lvl 5	Level 5 Service	
KEMH	Level 6 Service	
Paed Reg	Paediatric Registrar	Paediatric Registrar
Paed Con	Paediatric Consultant	Paediatric Consultant

Level 5 FSH

Level 6 KEMH

	Severity or Examples	Ranking	Additional Referral
Demographics			
Maternal age	16-40 yr.	G	
	>40.	A	
	<16 yr.	R	STAR Clinic/MGP
Morbid obesity	BMI > 35	R	
	BMI >40	Lvl 5	
Low maternal weight	BMI < 20	A	
Medical & Surgical Complications			
Anaesthetic			
Condition that may cause anaesthetic difficulty	E.g. Neuromuscular disease, Scoliosis	A	+ Anaesthetics referral
Previous failure or complication of anaesthesia	E.g. difficult intubation, failed epidural	A	+ Anaesthetics referral
Gynaecological			
Cervical surgery including cone biopsy, laser excision or LLETZ	Medium: Previous LLETZ	A	
	High: Previous cold knife cone or uterine surgery + > x1 LLETZ	R	
Cervical surgery with subsequent term vaginal birth		G	
Cervical amputation		R	
Abnormal PAP smear results requiring follow up during pregnancy		R	
PAP Due		G	
Congenital abnormalities of the uterus	Without previous normal pregnancy outcome	R	
Infertility	IVF or GIFT pregnancy	A	
Previous uterine surgery	Myomectomy	R	
	Previous uterine perforation	R	
Prolapse	Previous surgery	R	

Pelvic floor reconstruction	Previous surgery - colpo-suspension following prolapse, fistula and/or previous rupture	R	
Uterine fibroids	Medium: Uncomplicated fibroids	A	
	High: Cervical fibroid	R	
	Tertiary: Cervical, lower segment, multiple >5cm, pain increasing size on ultrasounds	R	
Vaginal Abnormality	E.g. Septum	R	
Female Genital Mutilation (FGM)		R	
IUCD in situ		A	
Pelvic deformities	trauma, symphysis rupture, rachitis, known bone pelvic deformity	R	
Gynaecological anatomical abnormality with recurrent pregnancy loss		R	
Surgical			
Abdominal trauma	Medium: Any Trauma/Surgery in current pregnancy	R	
	High: Previous pelvic fracture	R	
Breast surgery		G	
Appendicitis	Past history	G	
	Current	R	
Surgery during pregnancy		R	+ Anaesthetics Referral
Cardiovascular			
Cardiac	Arrhythmia, Cardiac valve disease, Ischaemic heart disease Cardiomyopathy, Congenital cardiac disease	R	+Cardiology input, + Anaesthetics Referral
Hypertension (incl essential, endocrine, renal)	Hypertension	A	
	Eclampsia	R	+ Anaesthetics Referral
	Gestational (pregnancy induced hypertension (PIH)	A	
	Pre-eclampsia: BP of ≥140/90 and/or relative rise of > 30/15mmHg from BP at commencement of care and any of proteinuria > 0.3g/24 hours; or protein/creatinine ratio ≥ 30mg/mmol or 2+ protein on dipstick testing • Platelets < 150 x 10 ⁹ /l • Abnormal renal or liver function Imminent eclampsia	R	+ Anaesthetics Referral
Pulmonary hypertension		Lvl 5	
Thromboembolism	E.g. previous DVT, PE, VTE (Prophylactic anticoagulation)	R	+ Anaesthetics Referral
Cardiac surgery	Cardiac valve replacement	Lvl 5	
Dermatological			
General	Any requiring systemic medication	A	
Endocrine			
Diabetes	Pre-existing (insulin dependent or non-insulin dependent)	Lvl 5	
	Gestational, well controlled on diet	A	
	Gestational, requiring insulin	Lvl 5	
Thyroid disease	Addison's, Cushing's or other endocrine diseases requiring TX	R	+ Anaesthetics Referral
	Hypothyroidism (stable treated and new dx)	A	
	Hyperthyroidism	R	

	Other thyroid disease	R	
Hypopituitarism		Lvl 5	
Prolactinoma		R	+ Level 5
Gastroenterological			
Cholelithiasis (gallstones)		A	
Cholecystitis or biliary colic		R	
Cholestasis of pregnancy	Gestation > 34	A	
	Gestation < 34	R	
Inflammatory bowel disease	Non complicated	A	
	Complicated	R	
Chronic liver disease		R	+ Anaesthetics Referral
Hepatitis (Acute /chronic)	Chronic or Acute	R	+ Anaesthetics Referral
Oesophageal varices		Lvl 5	
Liver cirrhosis/failure		Lvl 5	
Genetic			
General	Any condition with which the practitioner does not have detailed familiarity	R	
Marfan's		R	+ Anaesthetics referral
Raynaud's disease and other systemic and rare disorders		R	+ Anaesthetics Referral
Haematological			
Anaemia	Hb < 90 g/l, not responding to treatment, Haemolytic anaemia, Macrocytic anaemia, Haemoglobinopathy incl Thalassaemia, HbE, Sickle cell disease	R	+ Anaesthetics Referral
	Iron deficiency	G	
	Thalassaemia minor, B12, folate	A	
	Haemolytic or major thalassaemia or sickle cell	Lvl 5	
	Jehovah's witness or decline transfusion	Lvl 5	
Haemolytic anaemia		R	+ Anaesthetics Referral
Bleeding disorders	Von Willebrand's disease	R	+ Anaesthetics referral
Haemophilia	Particularly women with a male fetus or unknown sex	KEMH	
Thrombophilia	Including Antiphospholipid syndrome, Anticardiolipin antibodies, Lupus anticoagulant, hereditary thrombophilia	R	+ Anaesthetics referral
Thrombocytopenia <150 (x10 ⁹ /L)		A	+ Anaesthetics Referral
Mean Corpuscular volume (MCV) <80		A	
Blood group incompatibility		R	
Rhesus negative requiring Anti D		G	
Women declining blood products		Lvl 5	
Infectious Diseases			
Possibility of acute or chronic Infection	Viral: Rubella, Varicella, Cytomegalovirus (CMV), Parvovirus, HIV, Hepatitis (A, B, C, D or E)	R	
	Bacterial: Syphilis, Tuberculosis, Listeriosis	R	

	Protozoan: Toxoplasmosis, Malaria	R	
	Genital Herpes: Late in pregnancy - active lesions	R	
	Genital Herpes: Primary infection	R	
	Genital Herpes: Recurrent (consider antivirals to begin at 36wks)	A	
	Gonorrhoea	A	
	Chlamydia	A	
	HPV	G	
	Listeriosis	R	
Iso-immunity			
Red Cell Iso-immunisation		R	+ referral to KEMH
Platelet Isoimmunisation (NAIT)		KEMH	
Neurological			
AV malformation, CVA, TIAs		Lvl 5	
Epilepsy	With medication or seizure in last 12 mths	R	+ Anaesthetics Referral
	W/o medication or TX and no seizures in last 12 mths	A	
Multiple sclerosis		R	+ Anaesthetics referral
Myasthenia gravis		R	+ Anaesthetics referral
Spinal cord lesion		R	+ Anaesthetics referral
Muscular Dystrophy or Myotonic Dystrophy		R	+ Anaesthetics referral
Bell's palsy		A	
Subarachnoid haemorrhage, aneurysms		R	+ Anaesthetics referral
Other neurological conditions		R	+ Anaesthetics referral
Psychiatric & Chemical Dependency & Perinatal Mental Health			
Alcohol or drug dependency	Alcohol or drug dependency history	A	
	All current drug or alcohol dependant and unstable condition	KEMH	WANDAS/STAR
Psychiatric condition	Severe psychiatric issues on treatment	R	
Mental health issue requiring medication		A	+ MHCLN Referral
EPDS > 12		A	
EPDS positive response to Q10 self-harm		A	+ MHCLN Referral
Puerperal psychosis	Historic	A	+ MHCLN Referral
	Current	Lvl 5	+ MHCLN Referral
Nephrological			
Glomerulonephritis, Pyelonephritis, Renal failure of insufficiency, renal abnormality or Vesico- ureteric reflux	Renal failure	Lvl 5	
Pyelitis		A	
Previous Kidney surgery with potential to impair kidney function during pregnancy	i.e. removal of kidney etc	R	
Chronic renal disease		Lvl 5	

Asymptomatic bacteriuria		A	
Respiratory Disease			
Asthma	Controlled asthma/bronchitis	A	
	Moderate - oral steroids on two occasions in last 12/12	A	
	Severe - hospitalisation in the last 2 years - any previous admission to intensive care - FEV1 < 70% in absence of acute attack, - requiring bronchodilator therapy daily - > 1200 mcg Budisonide or equivalent	R	+Lvl 5 consult
H1N1		R	+Lvl 5 consult
Sarcoidosis		R	+ Anaesthetics Referral
Smoker	Smoker > 11 per day	A	
Severe lung function disorder		R	+Lvl 5 consult
Rheumatological			
Connective Tissue Disease	incl SLE, Rheumatoid Arthritis, PAN, Scleroderma	R	+ Anaesthetics Referral
Skeletal			
History of developmental skeletal disorders		R	+ Anaesthetics Referral
Osteogenesis imperfecta		R	
Scheuermann's disease		A	+ Anaesthetics Referral
Scoliosis (with or without rods		A	+ Anaesthetics referral
Spondylolisthesis		A	+ Anaesthetics Referral
Symphysis pubis dysfunction		G	+ Physio Referral
Other			
Organ transplants			
Indications developed/discovered during pregnancy			
Antenatal Complications: Maternal			
Previous Antenatal care	Late antenatal booking	A	
	No prior prenatal care (at full term)	R	+Social Work Referral
Uncertain dates in 3 rd trimester of pregnancy		R	+Social Work Referral
Adoption - intended		A	+Social Work Referral
First trimester screen: amniocentesis	Medium: Abnormal HCG with normal amniocentesis & Low PAPP A <0.4MOM	A	
	High: Abnormal amniocentesis / abnormal NT	R	
Placenta praevia	Medium: low lying placenta <25mm at 18-20w scan	A	+ Anaesthetics Referral
	High: Confirmed at 32 w G1-2, low lying placenta and APH	R	+ Anaesthetics Referral
	Tertiary: G3-4 praevia, Previous C/Section and anterior placenta praevia	Lvl 5	
Placental abruption		R	+ Anaesthetics Referral
Vasa Praevia		R	+ Anaesthetics Referral

Placenta accreta		Lvl 5	
GBS positive		G	
Genital herpes active in late pregnancy or at onset of labour		R	
Hyperemesis Gravidarum		A	
Antepartum haemorrhage (APH)	APH > 12 wks. Review and plan	A	
	APH & Low-lying placenta at 18-20 wk. scan.	R	
Ectopic pregnancy		R	
Clinical Complications: Maternal			
Acute abdominal pain		R	+ Medical/ Anaesthetic
Palpitations	Recurrent, persistent or associated with other symptoms	A	+Anaesthetic referral
Hypertension	>= 140/90 or	A	+ Anaesthetics Referral
	relative rise >= 30/15	A	+ Anaesthetics Referral
Proteinuria	> 0.3g / 24 hours	R	
Haematuria		A	
Hernia Nuclei Puposi (slipped disc)		R	+anaesthetic referral
Potentially significant clinical presentations during pregnancy including acute Abdo pain, palpitations, neuro symptoms, intractable headache		R	+specialty input, + Anaesthetics Referral
Antenatal Complications: Fetal			
Uterine Fundus Small for dates	Fundal height ≤ 4 cm below expected for gestational age	A	
Uterine Fundus Large for dates	Fundal height ≥ 4 cm above expected for gestational age	A	
Oligohydramnios		R	
Polyhydramnios		R	
Reduced fetal movements	More than one presentation with decreased fetal movements and a normal CTG	A	
High head at term		A	
Fetal anomaly/Fetal abnormality		R	+/- SO, and KEMH
Macrosomia	> 90 th centile for GA	A	
IUGR	< 10 th centile for GA	A	
	< 3 rd centile for GA	R	
Pathological CTG		R	
Fetal distress		R	+ Anaesthetics Referral
Perinatal death		R	
Past Obstetric History			
Obstetric Complications			
Grand multiparity	>5	A	
Multiple pregnancy	Twins or higher order multiples	R	
	Twin-Twin Transfusion Syndrome	KEMH	
Recurrent 1 st miscarriage	2 or less first trimester miscarriages	G	
	3 or more	R	

Mid/late trimester miscarriage	1 or more	R	
Previous Termination of pregnancies	3 or more	A	
Previous placental abruption		R	
Preterm Delivery, Cervical incompetence, Preterm PROM	< 34	R	
	35-36.6 weeks'	A	
Previous Caesarean section	Previous LUSCS x 1 (to NBAC midwife)	G	+ Anaesthetics Referral
	Previous LUSCS x 2 (3rd booking)	A	+ Anaesthetics Referral
	Previous c/s with severe complications, Previous C-section >4	R	+ Anaesthetics Referral
	Previous C/Section and anterior placenta praevia	Lvl 5	+ Anaesthetics Referral
Previous Classical Caesarean section		R	+ Anaesthetics Referral
Previous Manual removal		A	
Previous Instrumental birth		A	
Previous Postpartum haemorrhage		A	
Previous Shoulder dystocia		R	
Previous complications of anaesthetic, other analgesia or sedation		R	+anaesthetic referral
Previous obstructed labour		R	
Previous 3rd & 4th degree lacerations		R	
Previous cervical laceration		A	
Previous cervical incompetence		R	
Postpartum haemorrhage (PPH)	PPH 500ml no intervention	G	
	PPH >500ml requiring intervention & PPH requiring blood transfusion	A	
	ICU admit for PPH	R	
Previous Retained Placenta		A	
Vaginal laceration		G	
Medical Complications			
Autoimmune thrombocytopaenia		R	+ Anaesthetics Referral + haematology consultant
Blood group antibodies		R	
ABO-incompatibility		R	+paediatric referral
Trophoblastic disease		R	
Hypertensive disease	Requiring antihypertensive Rx	A	
Pre-eclampsia	See ASSHP definition	R	+ Anaesthetics Referral
Eclampsia		R	+ Anaesthetics Referral
Gestational hypertension		A	
Malignancy		R	
Urinary Tract Infection (UTI)	Recurrent	A	
Herpes genitals		A	

Gestational diabetes in previous pregnancy	Previous GDM on diet following protocol	A	
	Previous GDM on insulin	R	
Cholestasis	Previous cholestasis in pregnancy Prev mild + abn LFT	A	
Placenta Accreta		Lvl 5	
Fetal / Neonatal Complications			
Red Cell Iso-immunisation	Including severe or persistent jaundice	KEMH	+paediatrics
Platelet Isoimmunisation (NAIT)	Including neonatal haemorrhage or excessive bruising	KEMH	
Rhesus Isoimmunisation		KEMH	
Macrosomia	> 90 th centile for gestational age	A	
IUGR	< 10 th centile for gestational age	A	
	< 3rd centile for gestational age	R	
Neonatal birth injury		R	
Perinatal death		R	
Fetal abnormality	Minor	A	
	Major	R	
Neonatal asphyxia	AGPAR <7 at 5 mins	A	
Previous baby with GBS		A	
Previous baby poor condition at birth		R	
Puerperal Complications			
Vulval and perineal haematoma		A	
Breast abscess		A	
Postnatal depression		A	+ Mental Health CLN Referral
Previous serious psychological disturbance		R	+/- tertiary referral & MH CLN Referral
Dyspareunia > 3/12 postpartum		A	
Urinary Incontinence > 6/52 postpartum		R	
Faecal Incontinence		R	
Prolapse		A	
Symphysis pubis dysfunction		G	